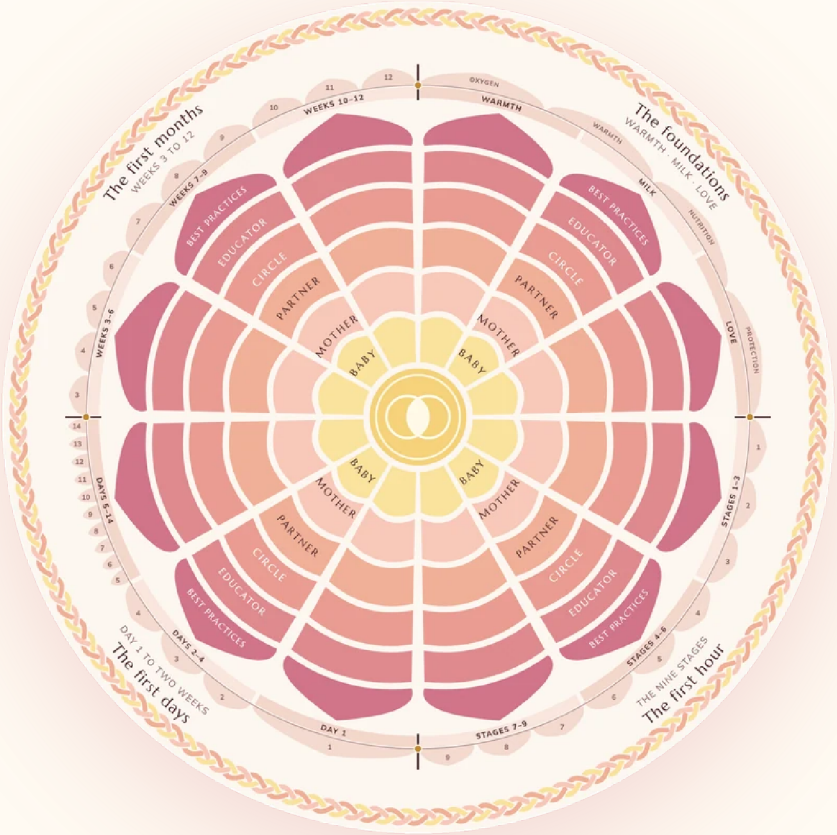
A WORKBOOK FROM THE FOURTH TRIMESTER MANDALA



My Fourth Trimester Plan

For the first hour, the first weeks, and the people who hold you

This plan belongs to _____

For my baby _____

Due _____

ERIN SINGLETON

WELCOME

Dear mother

So much happens after a baby arrives, and families often plan carefully for the birth and much less for what comes after. This workbook is a place to plan the fourth trimester as lovingly as the birth itself.

In these weeks your body is healing, your milk is coming in, sleep comes in pieces, and you are meeting a new person while becoming a new version of yourself. Your baby needs you close: your chest is her habitat, and your calm is her calm. You need holding too.

Here you can write down your first hour, the people in your circle and what you would love from each of them, how you plan to feed, the help that would let you rest, the signs that you might be struggling, and, when you are ready, your birth story.

How to use it

- [ ] Fill it in with your partner in the last weeks of pregnancy.
- [ ] Share it with your circle, so everyone knows how to hold you.
- [ ] Show your first hour and feeding plans to your midwife or doctor.
- [ ] Pin the Ways to help page where visitors will see it.
- [ ] Return to it as the weeks unfold. Plans can change, and that is all right.

You can also fill it in online, on your phone or computer, and print it with your own words written in.

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THE HEART OF THIS PLAN

Kangaroo care

Kangaroo care means holding your baby skin to skin on your bare chest, as much as you can, and keeping her close. It began in 1978 with doctors in Bogotá caring for small babies, and the research now shows it helps every newborn (Bergman; Moore et al., 2016).

Why it matters

A newborn's brain is only about a quarter developed at birth, and to her there are two places: mother, or other. On her mother's bare chest, within minutes, her heart rate and breathing settle. Her mother's chest warms by up to about 2°C (3.6°F) to warm her, or cools by about 1°C to cool her, and only the mother's chest adjusts this way. Oxytocin rises, the milk flows, and both stay calm. Apart, her body moves into defense: crying to bring her mother back, then shutting down to save energy (Bergman).

Three parts

- Skin to skin from birth, as continuously as you can.
- Breast milk from birth, then feeding on cue, day and night.
- Support for the mother and baby: whatever is needed, from whoever can give it.

Partners too

A baby on her partner's bare chest is kept warm and settles too, though only the mother's chest changes its temperature to match her. After a cesarean, babies held skin to skin by their fathers cried less and calmed sooner (Erlandsson et al., 2007).

Our song, and our calming playlist

How to hold her

- ○ Upright on your bare chest, chest to chest, between your breasts.
- ○ Her head turned to one side, her nose and mouth visible, her chin slightly lifted.
- ○ Her legs tucked up, like a little frog.
- ○ A blanket or your top over you both, not between you.
- ○ Stay awake, or have another adult watching. If you feel drowsy, hand her over or move her to her own safe space.
- ○ Stay for at least an hour at a time, since each move disturbs her (WHO).

Carrying her close

A wrap or soft carrier keeps her in her habitat while you move. Keep her snug and upright, high enough to kiss the top of her head, her face always in view, her chin off her chest, and her back supported.

My kangaroo care plan

- ○ Skin to skin from birth, as continuously as we can
- ○ Sessions of at least an hour, several times a day
  Each move in and out is a disruption for a small baby.
- ○ Skin to skin during feeds
- ○ My partner holds the baby skin to skin every day
- ○ A wrap or soft carrier, so she can be carried close
- ○ A warm room, a front-opening top, and a blanket over us both
- ○ Someone awake and watching whenever I feel drowsy

01 · 02

About me, and my circle of support

MY NAME

MY BABY'S NAME

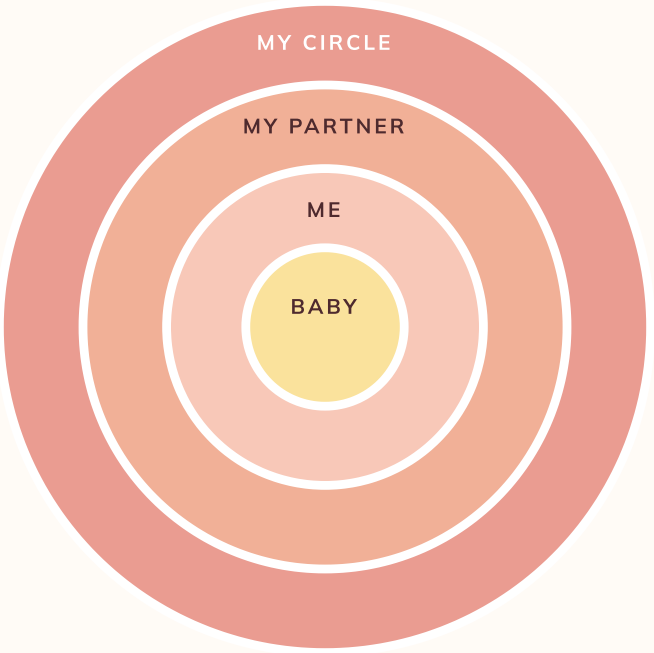
DUE DATE

WHERE I PLAN TO GIVE BIRTH

MY MIDWIFE OR DOCTOR

MY BABY'S CARE PROVIDER

Three words for how I want these weeks to feel



Each layer holds the one inside it, so no one carries it all alone.

Every mother and baby need at least two layers of support, and everyone who gives support needs support too (Castellino). Your partner, or your closest person, is the first layer. Around them are your main supporters, and around them the wider circle.

The people in my circle

A note to myself, for the weeks ahead

02

The people closest to me

My partner, or number one support

NAME

WHO THEY ARE TO ME

PHONE

WHEN THEY CAN HELP

Main supporter 1

NAME

WHO THEY ARE TO ME

PHONE

WHEN THEY CAN HELP

HOW THEY WILL HELP

Main supporter 2

NAME

WHO THEY ARE TO ME

PHONE

WHEN THEY CAN HELP

HOW THEY WILL HELP

Main supporter 3

NAME

WHO THEY ARE TO ME

PHONE

WHEN THEY CAN HELP

HOW THEY WILL HELP

The wider circle

Family, friends, a birth or postpartum doula, a lactation consultant, neighbors, a faith or parent community, a therapist.

WHO

NAMES

HOW TO REACH THEM

THEIR PART IN MY PLAN

WHO

NAMES

HOW TO REACH THEM

THEIR PART IN MY PLAN

WHO

NAMES

HOW TO REACH THEM

THEIR PART IN MY PLAN

WHO

NAMES

HOW TO REACH THEM

THEIR PART IN MY PLAN

03

For my partner

You are my first layer. The mother is the baby's habitat, and the partner protects and supports them both, so they can stay together (Bergman).

WHAT I WOULD LOVE FROM YOU

- ○ Keep the baby with me, skin to skin, as much as we can
  On my chest her temperature, heart, and breathing steady.
- ○ Guard the door: visitors, calls, and announcements wait until I am ready
- ○ Bring water and a snack every time I feed
- ○ Take night shifts for changing, settling, and bringing the baby to me
- ○ Hold the baby skin to skin on your own chest when I need to rest or shower
- ○ Listen to my birth story without fixing or correcting it
- ○ Watch my mood with me, and help me reach support if I need it
- ○ Tell me what I am doing well, often
- ○ Take care of yourself too, and tell me when you need support

In the first hour, I would love you to...

In the first days, please...

At night, I would like...

When I am feeding, please...

When I am overwhelmed or in tears, what helps most is...

Please protect me from...

Words I would love to hear from you

How I would like us to share caring for the baby

What you need, and who supports you

04

Requests for my circle

A card for each person or group. Kind, clear requests make it easier for people to say yes.

TO

WHAT I WOULD LOVE FROM YOU

WHAT WOULD NOT HELP RIGHT NOW

TO

WHAT I WOULD LOVE FROM YOU

WHAT WOULD NOT HELP RIGHT NOW

TO

WHAT I WOULD LOVE FROM YOU

WHAT WOULD NOT HELP RIGHT NOW

TO

WHAT I WOULD LOVE FROM YOU

WHAT WOULD NOT HELP RIGHT NOW

05

My first hour plan

Placed skin to skin and left undisturbed, a newborn moves through nine instinctive stages on her own (Widström). Share this page with your care team before the birth.

1	2	3	4	5	6	7	8	9
Birth cry	**Relaxation**	**Awakening**	**Activity**	**Rest**	**Crawling**	**Familiarization**	**Suckling**	**Sleeping**
at birth	right after	2–4 min	4–12 min	throughout	18–54 min	29–62 min	49–90 min	90–120 min

I WOULD LIKE

- [ ] My baby placed straight on my bare chest, dried there, with a warm blanket over us both
- [ ] Unbroken skin to skin for at least the first hour, until after the first feed
- [ ] Let my baby find the breast herself, with no one placing her on the nipple
  The breast crawl can take most of an hour; pauses are a stage of their own.
- [ ] Weighing, measuring, the bath, and routine procedures wait until after the first feed
- [ ] Newborn checks done on my chest, where possible
- [ ] Waiting before the cord is clamped
- [ ] Dim lights, quiet voices, and a calm room
- [ ] Visitors, calls, and announcements wait
- [ ] If I am drowsy or need care, my partner holds the baby skin to skin, or someone stays awake and watches her
- [ ] If I have a cesarean: skin to skin in the operating room if possible, or on my partner's chest until I am ready
- [ ] If my baby needs special care: my partner goes with her and keeps a hand and a voice with her
- [ ] Someone tells my baby what is happening, in a calm voice

The first words I would like my baby to hear

Notes for my care team

06

My feeding plan

Whatever path you choose, every feed can be close and calm, and skilled help early makes a real difference.

HOW I PLAN TO FEED

- ○ Breastfeeding
- ○ Pumping
- ○ Breast and bottle
- ○ Formula
- ○ Not sure yet

MY LACTATION CONTACT

THEIR PHONE

What I would like everyone to know

I WOULD LIKE

- ○ Skin to skin during feeds
- ○ Feed on cue, day and night, rather than by the clock
  Rooting, hands to mouth, and stirring come before crying.
- ○ A visit from a lactation consultant (IBCLC) in the first days
- ○ Help with latch and positioning before we leave the hospital or birth center
- ○ A calm minute before each feed: slow breaths, warmth, or music
  Calm helps the milk let down.
- ○ A breastfeeding group or La Leche League meeting
- ○ Ask me before offering the baby a bottle or a pacifier
- ○ If bottles are used: hold her close, let her set the pace, and switch sides

Visitors

Short, quiet, helpful visits protect your rest and your time with your baby.

- ○ Please text before you come
- ○ Please keep visits short, about an hour or less
- ○ Please wash your hands when you arrive
- ○ Please stay home if you are unwell
- ○ Please help with a task rather than hold the baby
- ○ Please do not kiss the baby's face

Who is welcome in the first days, and when

How and when we will share the news

06

Sleep and nights

Babies wake to feed often, day and night, and want to be close. A plan for nights lets everyone rest more and keeps the baby safe. Choose what feels right for your family, and talk it through with your provider.

WHERE MY BABY WILL SLEEP

- ○ In a bassinet or crib beside our bed, in the same room
  Room-sharing is recommended for at least the first six months (AAP).
- ○ In a bedside or side-car sleeper, close enough to reach
- ○ In our bed with me, for nursing through the night
  Many breastfeeding mothers choose this. The AAP advises against bed-sharing; if you plan it, learn the Safe Sleep Seven below and talk with your provider.
- ○ Skin to skin on a parent's chest, while that parent is awake
- ○ Not sure yet; I would like to learn more

For every sleep: on her back, on a firm, flat surface, with no pillows, loose blankets, bumpers, or toys, not too warm, in a smoke-free home. Falling asleep with a baby on a sofa or armchair carries the highest risk, so move somewhere safe, or hand her to someone awake, before you doze.

If I bed-share while breastfeeding

La Leche League's Safe Sleep Seven describes when it is safest:

- ○ You are a nonsmoker
- ○ You are sober and unimpaired
- ○ You are breastfeeding
- ○ Your baby is healthy and full-term
- ○ Your baby is on her back
- ○ Your baby is lightly dressed, not swaddled
- ○ You are both on a safe surface: a firm mattress, light covers, no gaps, away from pillows

Who helps at night, and how

Our plan for night feeds

How I will rest during the day

If I feel drowsy with the baby on my chest, she goes to...

07

Ways to help

A new mother's work is to rest, heal, feed, and be with her baby. Everything else can be shared.

Circle what you would like · nice to have · would love · most important

Care for the baby, close by

- Change diapers, then bring the baby back to me
- Burp and settle the baby after feeds
- Hold the baby skin to skin while I sleep or shower
- Carry the baby in a wrap or carrier for a short walk
- Bathe the baby, when we are ready
- Restock diapers, wipes, and clothes by the bed and the changing area
- Keep a simple log of feeds and diapers in the first week
- Learn the baby's early hunger cues, so you can bring her to me before she cries
- Drive us to the baby's checkups

Feeding

- Set up a feeding nest: pillows, water, snacks, a phone charger, a burp cloth
- Bring me water and a snack at every feed
- Wash and dry pump parts and bottles
- Bring the baby to me for night feeds
- Help me reach a lactation consultant or a breastfeeding group
- Prepare bottles, if we use them, the way I have asked

Meals and nutrition

- Organize a meal train
- Fill the freezer with meals before the birth
- Cook warm, nourishing meals: soups, stews, broths
- Make breakfast I can eat with one hand
- Keep a snack basket stocked: nuts, fruit, cheese, crackers
- Keep my water bottle filled beside me
- Plan the week's meals with my food notes in mind
- Wash up after meals

Groceries

- Make the grocery list with me
- Do the weekly shop, or order a delivery
- Pick up top-ups: milk, bread, fruit
- Put the groceries away
- Pick up pads, nipple cream, and other recovery supplies

Household and cleaning

- Tidy the main rooms
- Clean the kitchen
- Clean the bathroom
- Vacuum or sweep
- Change the bed sheets
- Take out the trash and recycling
- Water the plants
- Keep the house warm, and open a window for fresh air

Laundry

- Wash, dry, and fold laundry
- Wash the baby's clothes and blankets
- Put clean clothes away
- Keep clean towels and burp cloths stocked

Errands

- Pharmacy pickups
- Post office, packages, and returns
- Bills and banking
- Fuel the car and check the car seat
- Pick up anything else I need

Older children

- School or daycare drop-offs and pick-ups
- Playdates and outings
- Meals and snacks for them
- Help with homework
- Help at bedtime
- Special one-on-one time with each child

Pets

- Feeding
- Walks
- Litter or cleaning
- Vet visits

Care for my body

- Hold the baby while I shower or bathe
- Run a warm bath or a sitz bath
- Restock pads, ice packs, and comfort supplies
- Help me get up and to the bathroom in the first days
- Remind me to take any medication or vitamins
- A foot rub, a massage, or a hand on my back
- Help with my hair
- Drive me to my postpartum checkup
- Help me book a pelvic floor physiotherapist, if I need one

Rest and sleep

- Protect one long stretch of sleep for me each day
- Take a night shift
- Keep the bedroom dark and quiet
- Hold the baby skin to skin so I can nap
- Turn visitors away kindly when I am resting

Emotional care and connection

- A daily check-in: how are you, really?
- Listen to my birth story without fixing it
- Sit with me on hard nights
- Remind me that I am doing well
- Watch for signs I am struggling, and help me reach support
- Bring me outside for fresh air or sun
- Keep me company, without needing me to host

Visitors and messages

- Share the news and updates for us
- Answer messages and calls
- Schedule visits and keep them short
- Remind visitors to wash their hands, and to stay home if unwell
- Keep track of gifts and thank-you notes

Paperwork

- Register the birth and order the birth certificate
- Add the baby to our health insurance
- Parental leave and work forms
- Keep a calendar of appointments and bills

My own requests

-
-
-
-
-

08

Our help plan

Your most important requests, and who has said yes. Most important first.

PRIORITY	WHAT	WHO	WHEN
◊ ◊ ◊			
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08

For each of my supporters

A card for each person, with the help you have asked of them. Cut them apart, or share a photo.

NAME PHONE

THE HELP I AM ASKING OF YOU

NAME PHONE

THE HELP I AM ASKING OF YOU

NAME PHONE

THE HELP I AM ASKING OF YOU

NAME PHONE

THE HELP I AM ASKING OF YOU

NAME PHONE

THE HELP I AM ASKING OF YOU

NAME PHONE

THE HELP I AM ASKING OF YOU

09 · 10

Rest, recovery, and watching over me

Kimberly Ann Johnson names five needs of every new mother, and many cultures keep a lying-in of about forty days. Write what each means for you.

HOW LONG I WOULD LIKE TO REST AND BE CARED FOR

Extended rest

Nourishing food

Loving touch

Wise women around me

Time in nature

Anything else I need

Watching over me

The baby blues are common and usually pass within two weeks. Sadness, worry, frightening thoughts, rage, or numbness that last or deepen may be postpartum depression or anxiety: common, treatable, and not her fault.

Signs that I am struggling

What helps me come back

Who to call if you are worried about me

When to call right away. For the mother: heavy bleeding (soaking a pad in an hour or less), a fever, a severe headache or changes in vision, chest pain or trouble breathing, a swollen, painful leg, or thoughts of harming herself or the baby. For the baby: a fever (under three months, 100.4°F or 38°C), trouble breathing or a bluish color, being very hard to wake, refusing feeds, far fewer wet diapers, or yellow skin that is spreading.

National Maternal Mental Health Hotline (US), free, day and night: 1-833-TLC-MAMA (1-833-852-6262) · Postpartum Support International: postpartum.net

11

My birth story

Telling the story of a birth, and being heard, is part of healing (Kendall-Tackett). Write as much or as little as you like, whenever you are ready. There is no right way for a birth to have gone.

How it began

How my baby was born

Who was with me, and what they did

What was hard

What I am proud of

What I would like my circle to know

What I would like to tell my baby about her birth

My partner's story

12

For my supporters: why this matters

The key ideas behind the requests in this workbook, so you know why closeness and quiet matter so much right now. The ideas behind them are gathered in Key concepts on page 18, and there is more in The Fourth Trimester Mandala.

Her chest is the baby's habitat

A newborn's brain is only about a quarter developed at birth, and to her there are two places: mother, or other (Bergman). On her mother's bare chest, her heart rate and breathing settle within minutes, and her mother's chest warms by up to about 2°C to warm her or cools by about 1°C to cool her. Only the mother's chest adjusts this way. Apart, the baby's body moves into defense: protest, then despair, with stress hormones high. This is why we ask you to keep them together, and to help with everything else.

The first hour

Left skin to skin, a newborn moves through nine stages on her own: her birth cry, relaxation, awakening, rooting, rest, the crawl to the breast, licking and familiarizing, latching, and a deep sleep (Widström). She knows the way. Please wait, keep the room quiet, and let routines and visitors wait.

Milk, and why calm matters

Human milk is light, the milk of a baby meant to be carried and fed often, about every two hours or whenever she asks (Bergman). Stress hormones can slow the milk; calm, warmth, and support help it flow (Jayamala; Feher). Protecting the mother's rest and peace is protecting the baby's food.

Hormones and co-regulation

Oxytocin, the hormone of love and calm, flows with skin to skin, touch, and feeding, and it is held back by fear, pain, cold, and feeling watched or rushed (Uvnäs Moberg; Buckley). A newborn borrows her mother's nervous system to settle, and a mother settles more easily when she is held. Your calm counts: you steady the mother, and she steadies the baby. No one is meant to be the calm one alone.

Her language

Crying is a baby's first spoken language, and around it is a language of the body. Feeding cues come early: stirring, rooting, hands to mouth. Looking away, yawning, hiccups, or splayed fingers mean she needs a pause. Some crying is about needs now, hunger, cold, tiredness; some may be about her birth. Meet her needs first, then hold her, stay calm, and say what you see (Appleton).

How to support

- Do the tasks so the mother can hold the baby, rather than holding the baby so the mother can do the tasks.
- Keep visits short, quiet, and helpful.
- Listen to her birth story without correcting or comparing.
- Tell her what she is doing well.
- Watch for signs she is struggling, and help her reach care early.

If you are caring for the baby

- Hold her close, skin to skin on your bare chest if you can, with her face visible and her nose and mouth clear.
- Tell her who you are and what you are doing, in a calm, low voice.
- Watch her cues, and bring her to her mother at the first signs of hunger.
- If you give a bottle, do it as her mother has asked: hold her close and upright, let her set the pace, and switch sides.
- For sleep: on her back, on a firm, flat surface of her own, with no pillows, blankets, or toys, in the room where her parents sleep.
- Return her to her mother as soon as she can be together with her, and let her mother know how the time apart went.

13

Important contacts

MY MIDWIFE OR DOCTOR

MY BABY'S CARE PROVIDER

HOSPITAL OR BIRTH CENTER

LACTATION CONSULTANT

BIRTH DOULA

POSTPARTUM DOULA

THERAPIST OR COUNSELOR

SOMEONE I TRUST

In the United States: emergency 911 · crisis line 988 · National Maternal Mental Health Hotline 1-833-TLC-MAMA · Postpartum Support International postpartum.net

Caring for your baby: key notes

- **Feeding:** eight or more times in 24 hours in the early weeks, on cue: stirring, rooting, hands to mouth come before crying.
- **Diapers:** by about day five, around six or more wet diapers a day, and regular stools. Ask your provider what to expect day by day.
- **The cord:** keep the stump clean and dry; it usually falls off within one to three weeks. Sponge baths until then.
- **Holding:** support her head and neck, keep her close, and skin to skin often.
- **Her states:** quiet alert, eyes bright and still, is her best time to connect. Looking away, yawning, or hiccups mean she needs a pause.
- **Crying:** meet her needs first, then hold her, stay calm, and say what you see. Crying often peaks around six weeks. At your limit, lay her down somewhere safe and breathe.
- **The car seat:** rear-facing, for every ride.
- **Sleep** and **when to call** are on pages 10 and 14.

Notes

REFERENCE

Key concepts

The named ideas behind this plan and The Fourth Trimester Mandala, gathered in one place: where each comes from, what it is, and why it matters.

The fourth trimester itself

The fourth trimester

From: The name is often credited to Sheila Kitzinger and was made popular by Harvey Karp.

The first twelve weeks or so after birth, seen as a continuation of pregnancy. Human babies are born very immature, so gestation goes on outside the womb, on the mother's body.

- The medical postpartum of six weeks
- ACOG's twelve weeks of ongoing care
- The lying-in of about forty days kept in many cultures

Why it matters. It frames these weeks as a time for closeness, rest, and support, not for getting back to normal.

In the mandala: About this mandala, Closing the fourth trimester

The primal period

From: Michel Odent and his Primal Health Research database.

The stretch from conception to the first birthday, when experience shapes lifelong health most deeply.

Why it matters. It places the fourth trimester inside a larger sensitive time that runs from pregnancy through the first year.

In the mandala: About this mandala

The baby's questions and possible imprints

From: The PPN view that a baby is aware and remembering from the start (Chamberlain; Verny), in the format of the Pregnancy and Birth Mandala.

Each moment holds an overall question the baby may be asking. How it is answered, yes or not yet, may leave an imprint.

Why it matters. It lets parents see the moment from the baby's side. Imprints are possibilities, not certainties, and every repair keeps answering the question.

In the mandala: The baby ring, in every slice

Kangaroo care and the habitat

Habitat and niche

From: Nils Bergman.

The habitat is where a newborn belongs, her mother's body. The niche is what that place makes possible: breastfeeding, sleep, and connection.

Why it matters. The two belong together. Take her out of her habitat, and her body and behavior change.

In the mandala: Foundations, Warmth, Milk

The four needs of every newborn

From: Bergman.

Oxygen, warmth, nutrition, and protection. In the womb the placenta meets them; after birth, her mother's chest does.

Why it matters. Skin to skin meets all four at once.

In the mandala: The time wheel in the Foundations quarter

Warmth, milk, and love

From: Edgar Rey and Héctor Martínez, the doctors in Bogotá who first described kangaroo care in 1978.

The three parts of kangaroo care. Warmth is the habitat, milk is the niche, and love is the setting that holds them.

Why it matters. It is the simplest summary of what a newborn needs, and it shapes the mandala's Foundations quarter.

In the mandala: Warmth, Milk, Love

Kangaroo mother care: three parts

From: Bergman's definition.

- Skin to skin from birth, as continuously as possible
- Breast milk from birth, then exclusive breastfeeding
- Support for the mother and baby: whatever is needed or available

Why it matters. It turns kangaroo care into something families and care teams can plan for.

In the mandala: Warmth · In this plan: Kangaroo care

The six stages of mammal life

From: Bergman, drawing on Rosenblatt.

Conception, gestation, parturition (labor), transition from womb to air, initiation of feeding, and maintenance of feeding.

Why it matters. Every stage leads to one shared endpoint, breastfeeding. The first hour is transition and initiation.

In the mandala: Arrival

The three programs of the hindbrain

From: Bergman.

Defense, nutrition, and reproduction. Only one runs at a time, and each sets the hormones, nerves, and muscles of the whole body.

Why it matters. On her mother's chest, nutrition and growth switch on. Apart, defense takes over.

In the mandala: Warmth

Kangaroo care and the habitat, continued

Protest and despair

From: Bergman, from research across mammals.

The response to separation. First protest, crying to bring her mother back. Then, if no one comes, despair: she withdraws, her temperature and heart rate drop, and stress hormones rise to save energy for survival.

Why it matters. A baby who goes quiet when apart may be shutting down, not settling.

In the mandala: Warmth, The crying weeks

Thermal synchrony

From: Bergman; Ludington-Hoe and colleagues.

A mother's breast warms by up to about 2°C to warm a cool baby, and cools by about 1°C to cool a warm one. With twins, it responds to each twin. Only the mother's chest does this.

Why it matters. It is one reason the mother's chest is a baby's first place.

In the mandala: Warmth

The first hour

Widström's nine stages

From: Ann-Marie Widström and colleagues.

The instinctive stages a newborn moves through when left skin to skin: Birth cry · Relaxation · Awakening · Activity · Rest · Crawling · Familiarization · Suckling · Sleeping.

Why it matters. She knows the way. Not placing her on the nipple, and waiting with routines, protects the sequence.

In the mandala: The First hour quarter and its time wheel petals

Reading the baby

The six states

From: Marshall Klaus; T. Berry Brazelton.

Quiet sleep, active sleep, drowsy, quiet alert, active alert, and crying.

Why it matters. Each state has its own look and needs. Quiet alert is her best time to connect.

In the mandala: Her language, Day 1

Needs crying and memory crying

From: Matthew Appleton.

Needs crying is about the present: hunger, cold, tiredness. Memory crying relives part of the birth, with anger, anxiety, or grief, and may grow louder before it settles. *Good Baby Syndrome* is a baby who has stopped expecting to be heard.

Why it matters. Meeting needs first, then listening, helps a baby release what she carries.

In the mandala: The lying-in, Her language

Baby body language

From: Appleton.

Fixed signs (conjunct sites, conjunct pathways, the birth lie side) and active signs (touching or tracing the same spots).

Why it matters. A baby's body can show where birth pressed on her, and naming it gently can help her settle.

In the mandala: Milk comes in, Going deeper readings

The crying curve

From: Ronald Barr.

In healthy babies, crying rises from about two weeks, peaks around six to eight weeks, then eases. More carrying means less crying (Hunziker and Barr).

Why it matters. It is a normal stage, not a sign anyone is doing something wrong.

In the mandala: The crying weeks

Bonding and the nervous system

The hormones of bonding

From: Sarah Buckley; Kerstin Uvnäs Moberg.

Oxytocin (love and calm), prolactin (milk and mothering), endorphins (comfort and pleasure), and dopamine (reward), with adrenaline and cortisol as the stress hormones.

Why it matters. These hormones flow with safety and closeness, and fear and stress hold them back.

In the mandala: Co-regulation

Babies borrow your brain

From: Greer Kirshenbaum.

A newborn cannot calm herself yet, so she borrows her parent's calm. Each time, the *nurture bath* replaces stress hormones with oxytocin. Your calm is the bridge back to safety, and she crosses it in her own time.

Why it matters. Co-regulation, not self-soothing, is how babies come back to calm.

In the mandala: Co-regulation

The heart's field and coherence

From: Appleton.

A calm, loving mother's heart rhythm may draw her baby's into step, most strongly within about three feet. This is still being researched.

Why it matters. The mother's own calm is part of what steadies her baby.

In the mandala: Love

Bonding and the nervous system, continued

The First Year Attachment Cycle

From: Vera Fahlberg; applied to birth by Appleton.

Need, then arousal, then the need met, then relaxation, then trust.

Why it matters. Repeated thousands of times in the first year, it builds her sense that the world is safe.

In the mandala: Love, Her language

Nurtured empathy

From: Kirshenbaum.

Name what she is doing, what she might feel, and what she might need, then meet the need.

Why it matters. It lends her your words for what she feels, and on hard days simply holding her and saying "I'm here" is enough.

In the mandala: Love

Growing SPACE

From: Kirshenbaum.

Self-awareness, Pause, Aware of the sensations, Create movement, Emotion processing.

Why it matters. It is a way for a parent to come back to calm before responding.

In the mandala: Co-regulation

Holding the mother

Castellino's layers of support

From: Ray Castellino (in White, 2013).

Every mother and baby need at least two layers of support, and everyone who gives support needs support. His principles: Mutual Support and Cooperation, Choice, the Pause, Self Care, Brief Frequent Eye Contact, Touch and Attention, and Confidentiality.

Why it matters. It is the reason the mandala has partner and circle rings.

In the mandala: The partner and circle rings

The five needs of every new mother

From: Kimberly Ann Johnson.

An extended rest, nourishing food, loving touch, the company of wise women, and time in nature.

Why it matters. It gives the lying-in its shape, and the circle a list of what to offer.

In the mandala: The lying-in

The Safe Sleep Seven

From: La Leche League.

The conditions under which bed-sharing is safest for a breastfeeding mother:

- A nonsmoker
- Sober and unimpaired
- Breastfeeding
- A healthy, full-term baby
- The baby on her back
- The baby lightly dressed
- A safe surface

Why it matters. It supports families who bed-share, alongside the AAP's advice to room-share without bed-sharing.

In this plan: Sleep and nights

Healing and repair

When stress becomes trauma

From: Appleton.

Stress becomes trauma when it overwhelms the ability to cope: in *hyper-arousal* (fight or flight) or *hypo-arousal* (freezing, shutting down). Unresolved, it may become a *super conductive survival system*, where later stress sets off the old survival response.

Why it matters. What turns a hard birth into lasting trauma is often a lack of empathy for what the baby went through, and being understood can change that.

In the mandala: The lying-in

Rupture and repair

From: Edward Tronick; Karen Strange.

Even close, loving pairs are often out of step, and they find their way back again and again. The still-face research showed how a baby suffers when her parent goes blank, and how quickly she recovers when the connection returns.

Why it matters. Repair is the point. Each return teaches her that connection comes back.

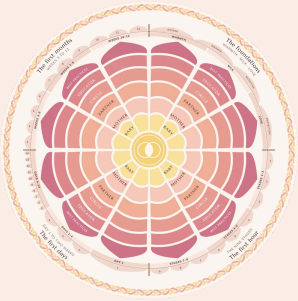
In the mandala: The repair in every slice, First smiles

My notes

NOTES

My notes

No one is meant to carry these weeks alone. Hold your baby; let your circle hold you.



A companion to The Fourth Trimester Mandala, a teaching map of the first twelve weeks after birth. erinsarita.github.io/fourth-trimester-mandala

Created by Erin Singleton, a student in APPPAH's Prenatal & Perinatal Educator certification. An educational workbook, not medical advice.